

Patient ID: JonesPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 704/745h (94%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01

■ HR ■ Breathing

Low Heart Rate May 08 to May 08 (1d)	Low Heart Rate May 11 to May 14 (4d)	Low Heart Rate May 19 to May 22 (4d)	Low Heart Rate May 25 to May 25 (1d)
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Last 24 Hours

Vital	Avg	Min	Max
Heart Rate	55 bpm	39 bpm	74 bpm
Breathing	17 brpm	12 brpm	27 brpm

9 episodic events Detected, spanning 12h total. Conditions: Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
9	12h	Low Heart Rate	<1	94%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 08	8 PM to 10 PM	2h	1	Low Heart Rate	43	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 11	3 AM to 4 AM	4h	3	Low Heart Rate	43	39	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 19	4 PM to 5 PM	5h	4	Low Heart Rate	44	41	51	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 25	12 AM to 1 AM	1h	1	Low Heart Rate	44	39	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

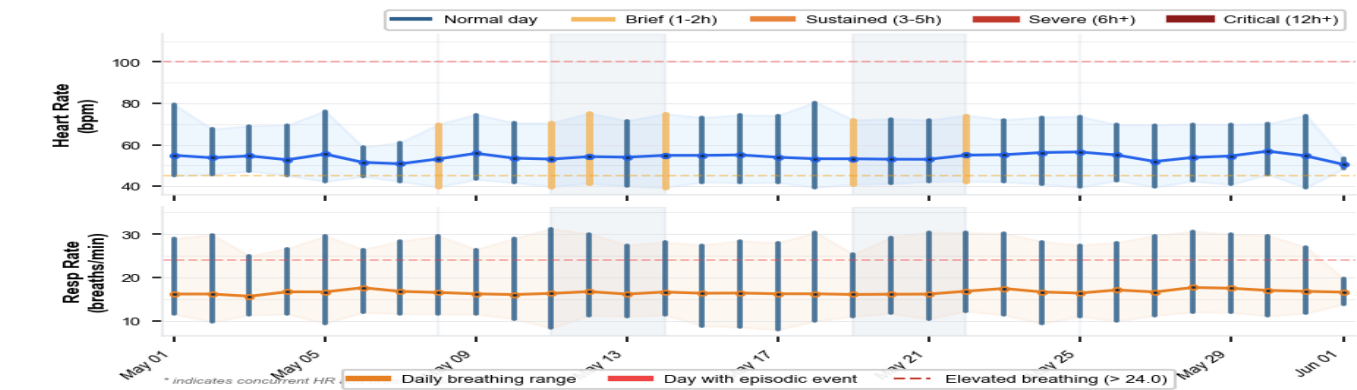
Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 4 unstable phases with 16 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

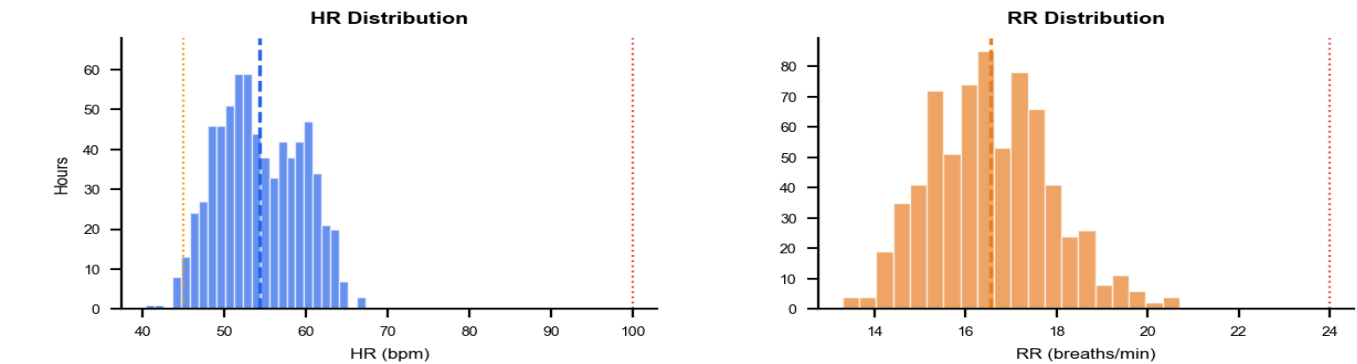
Clinical Pattern Observations:

- 1. Clustered pattern: Episodic events on 6 of 32 days (18%).
- 2. Nocturnal pattern: 4 of 6 eligible episodes during evening or night hours.

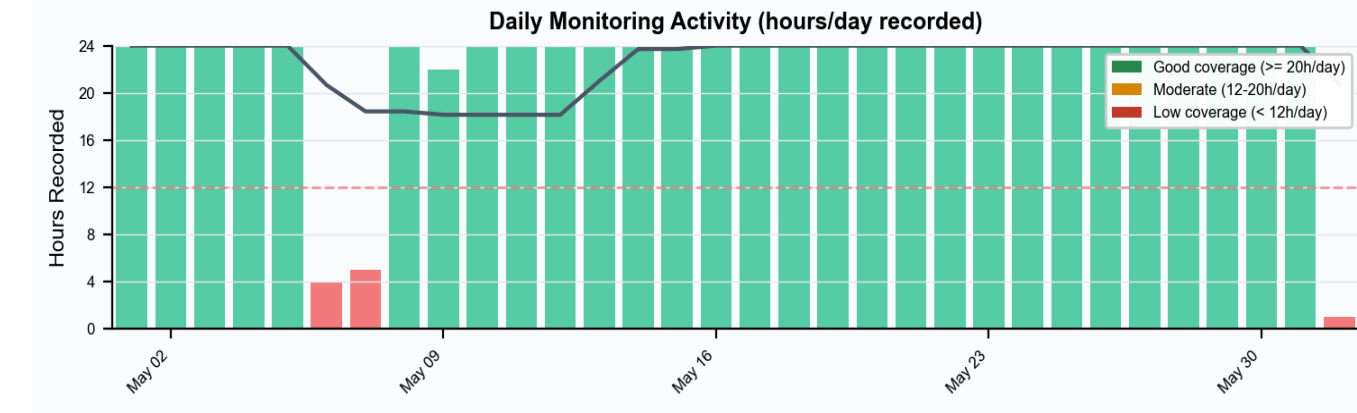


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.